F A X S H E E T

Date:	03/30/2026 12:43:41 AM
To:	STAR
Subject:	Referrals
Fax Number:	4809560654
To Company:	
From Name:	Euroza,Nancy
From Company:	Central Phoenix - Innovative Pain and Wellness
From Facility:	Central Phoenix - Innovative Pain and Wellness
Support Contact:	480-467-2273
Number of Pages(s):	11

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REFERRAL

Jina H Naaim, PA-C
Pain Medicine
Central Phoenix - Innovative Pain and Wellness
1277 E MISSOURI AVE STE 102, PHOENIX, AZ-85014-2916
Tel: 480-467-2273 Fax: 602-547-6887

OLIVIER BIZIMANA
08/12/1982

Date: 03/30/2026

Patient Information:

Patient Name: OLIVIER BIZIMANA
Patient DOB: 08/12/1982
Patient Insurance: Medicare Part B
Patient Subscriber No: 1Y63RT9JR08
Patient Address: 8330 N 19TH AVE, APT 2056, PHOENIX, AZ 85021-5287 US
Patient Phone: 623-227-6352
Patient Work Phone:
Patient Cell Phone: 623-227-6352
Patient SSN: XXX-XX-1686

Insurance Information

Insurance Name: Medicare Part B
Subscriber Name: BIZIMANA, OLIVIER
Subscriber DOB: 08/12/1982
Subscriber No: 1Y63RT9JR08
Subscriber Group No:
Subscriber Address: 8330 N 19TH AVE, APT 2056, PHOENIX, AZ 85021-5287 US
Subscriber Phone: 623-227-6352

Referral From Information:

Provider Name: Naaim, Jina, H
Provider ID Number:
Provider UPIN:
Provider NPI: 1952128381
Provider Facility: Central Phoenix - Innovative Pain and Wellness
Provider Speciality: Pain Medicine
Address1: 1277 E MISSOURI AVE
Address2: STE 102
City, State, Zip: PHOENIX, AZ, 85014-2916
Phone: 480-467-2273
Fax: 602-547-6887

Referral To Information:

Provider Name:
Provider ID Number:
Provider UPIN:
Provider NPI:
Provider Facility:
Provider Speciality:
Address1:
Address2:
City, State, Zip: , ,
Phone:
Appt. Date/Time:
Fax:

Facility Tax ID Number:

REFERRAL

Jina H Naaim, PA-C
Pain Medicine
Central Phoenix - Innovative Pain and Wellness
1277 E MISSOURI AVE STE 102, PHOENIX, AZ-85014-2916
Tel: 480-467-2273 Fax: 602-547-6887

OLIVIER BIZIMANA
08/12/1982

Reason For Referral:

Authorization No:	**Authorization Type:**
Reason:	AT HOME STAR PHYSICAL THERAPY- eval and tx for lumbar radiculopathy.
Diagnosis:	M54.16 - Lumbar radiculopathy
E/M Codes:	
Procedures:	
Visits Allowed:	99
Unit Type:	V (VISIT)
Start Date:	03/30/2026
End Date:	03/30/2027
Priority:	Routine

Provider NPI: 1952128381

Electronically signed by Jina H Naaim, PA-C on 03/30/2026 at 12:43 PM MDT

Progress Note

Progress Notes

Patient: BIZIMANA, OLIVIER
Account Number: 34131
DOB: 08/12/1982 **Age:** 43 Y **Sex:** Male
Phone: 623-227-6352
Address: 8330 N 19TH AVE, APT 2056, PHOENIX, AZ-85021-5287

Provider: Jina H Naaim, PA-C
Date: 03/30/2026

Subjective:

Chief Complaints:
1. MRI REVIEW.

HPI:
1. History of Problem::
Reason for today's visit? Back pain. Location of Pain? Lumber, thighs, legs and feet. Does you pain radiate? If so where? The pain radiates from the lumbar spine to the feet. What is your current pain on a scale of 0-10? 8. What is your average pain on a scale of 0-10? 6. Has your pain changed since last office visit? No. What is the frequency of your pain? Constant. Describe your pain: Aching,Burning,Tingling,Numbness. What makes your pain worse? Standing,Sitting,Walking,Twisting,Exercises. What makes your pain better? Laying down,medications.
2. Medication History::
How much pain relief/functional improvement does your current medication provide? 10-30%. Do you feel that you can use less medication than currently prescribed? No. Are you having any side effects from your medication? Yes. When was the last time you took pain medication? I have been out of medication.
3. Treatment History::
Are you currently taking Anticoagulants/ Blood Thinners? No. What doctor manages the blood thinner? None. Why are you taking a blood thinner? None. Have you had any procedures to treat your pain in the last two months? No. If yes, how much pain relief did you have? NA. Did you have any complications from the procedure? No. Are you currently attending conservative treatment including PT/Chiro/Massage/Acupuncture? If so which one? No. How many sessions have you completed? NA. Percentage of relief obtained from conservative treatment? 0%.
4. Diagnostic Studies::
Have you completed any Imaging or Diagnostic studies since last office visit? No.

ROS:
General / Constitutional:
Change in appetite denies. Chills denies. Weight gain denies. Weight loss denies. Weakness denies.
Respiratory:
Shortness of breath denies.
Cardiovascular:
Chest pain denies. Palpitations denies.
Gastrointestinal:
Constipation denies. Diarrhea denies. Nausea denies. Vomiting denies.
Hematology:
Bleeding problems denies.
Neurologic:
Dizziness denies. Fainting denies. Gait abnormality denies.

Medical History: No medical problems.

Surgical History: lumbar discectomy at L5 8/17/2022, back surgery 05/2025, Lumbar laminectomy LEFT L4 and L5 05/2025.

Hospitalization/Major Diagnostic Procedure: fall 2015, denies recent 03/2023.

Family History:
no family hx.

Screening:

Social History:
Migrated Social History: Drugs/Alcohol: (Alcohol Screen):No Did you have a drink containing alcohol in the past year?: No, Points: 0, Interpretation: Negative. Tobacco Use: (Tobacco Use/Smoking):No Are you a: : nonsmoker.

Medications: Taking Gabapentin 300 MG Capsule 1 capsule Orally three times a day , Notes to Pharmacist: Do not dispense until 30 days after last fill, or on the last business day prior to 30 days in cases where pharmacy is closed on

30th day. Don't drink alcohol while taking this medication.

Objective:

Vitals:

Wt: **230** lbs, Ht: **72** in, BP: **130/76** mm Hg, HR: **60**, Oxygen sat %: **96** %, Pain scale: **8** 1-10, Wt-kg: 104.33, BMI: **31.19**, Body Surface Area: 2.3.

Examination:

General Examination:

General: Awake, alert, and oriented
HEENT: atraumatic, mucous membranes moist
Respiratory: Normal respiratory excursion
Cardiovascular: Well perfused, no jvd
Extremities: No clubbing, Cyanosis or edema
Skin: Warm, dry, intact. no rashes on exposed skin.
MSK: Chronic arthritic changes in multiple joints

Back Exam:
Inspection: no muscle atrophy, reversal of normal lordosis and exaggeration of thoracic kyphosis
Palpation: TTP bilateral facet joints and paraspinal musculature
ROM: limited especially lateral rotation, pain on flexion and lateral rotation
Facet loading: RIGHT- positive/ LEFT- positive.

Assessment:

Assessment:

1. Lumbar radiculopathy - M54.16 (Primary)
2. Spondylosis of lumbosacral joint without myelopathy - M47.817
3. Neuropathy - G62.9
4. Long term (current) use of opiate analgesic - Z79.891

History- He incurred pain on August 15, 2015, he slid and fell on the floor when he was mopping. He began to feel numbness and weakness to his legs. He saw pain management and had two epidural injections. This did not help. He did physical therapy in 2015 and repeated again this year, the month prior to this initial appointment. He had discectomy on 8/17/22 at the back institute in Los Angeles. He was searching online and found this surgeon and had great testimonies. Per Dr. Jagadesh, no surgery indicated. He has more pain now. He cannot stand for longer than 10 minutes whereas before he could for 30 minutes. He is not able to work.

I reviewed CT of the lumbar spine performed on August 16, 2022 which revealed mild posterior disc bulge at multiple levels. There is mild facet arthropathy from L1-L5. There is moderate canal stenosis at L5-S1 with mild to moderate bilateral neural foraminal narrowing. At L4-5 there is severe central canal stenosis and moderate bilateral neural foraminal narrowing. L3-4 there is severe central canal stenosis as well as at L2-3. Reviewed an x-ray of the lumbar spine which revealed no evidence of fracture. EMG that was performed in 9/30/2022 was normal per patient.

It was recommended that he have a neurological consultation since he has not had relief from spinal injections and was not a surgical candidate. There may be a neurological condition that may be causing his pain. Pt did have neurology follow up and had a new LE EMG. Per report he is positive for L L3-4 radiculopathy. This was not identified on his last EMG.

Today:
patient is a pleasant 43-year-old male who is presenting for lumbar radiculopathy down the back of both legs. He reports that he did have a left L4 and L5 laminectomy in Africa about a year ago. Reports that he continues to have pain down both legs. Discussed lumbar MRI in detail today with patient. He would like to proceed with an epidural. He reports no surgeries at L5 and S1. Discussed ILESI L5/S1 in detail with patient. He would like to proceed. Denies any use of blood thinners. Reports that he would like to trial physical therapy. He would also like to hold off any surgical evaluations. Denies neurological deficits, red flag symptoms, or severe pain. ER precautions given. He would like a Toradol injection today due to pain. He reports that pain is worsened with standing for long period of time, walking for long period of time, sitting to standing positions as well. No pain in the buttocks or SI joint. Reports that he cannot do much for over 30 minutes before having pain. Discussed conservative measures and activity modification in detail as well. He would like to have increase in gabapentin. he would like to trial 400 mg 3 times daily first before increasing any further. Declines LSO today.

The patient presents with chronic *BACK*pain radiating into the **lower** extremity in a dermatomal distribution consistent with [L5/S1] radiculopathy. Pain has been present for approximately > 6 weeks. Physical examination demonstrates objective evidence of nerve root involvement, including + straight leg raise reproducing radicular symptoms, decreased sensation in the corresponding dermatome MRI of the lumbar spine dated shows stenosis at L4-5 and L5-S1 with impingement of the nerve roots, correlating with the patient's symptoms and exam findings. The patient has participated in and remains engaged in conservative management, including a structured physical therapy program from (core stabilization, stretching, and functional restoration) and an ongoing home exercise program performed at least 2 times per week for 6 weeks as well as trials of medications such as NSAIDs, neuropathic agents, activity modification, and use of heat/ice, without adequate or durable relief. Despite this conservative care over more than [6] weeks, the patient continues to experience function-limiting radicular pain that interferes with [walking/standing/sitting/sleep/ADLs]. An epidural steroid injection is requested as a medically necessary next step to reduce nerve root inflammation, improve pain, and facilitate further participation in active rehabilitation. The procedure will be performed under continuous fluoroscopic guidance with non-ionic contrast dye to confirm correct epidural placement and spread prior to steroid injection, in accordance with safety and Medicare requirements.

Diagnostic Testing:
-Lumbar MRI 03/2026 on chart and reviewed with patient in detail.
MRI L/S 4/18/24 attached in pt's chart

MRI L/S 10/4/22: Modic changes noted, full report attached in pt chart.

MRI L/S 10/12/22 (rad impression): L5-S1 broad-based disc protrusion 4 mm with annular visual tear epidural space. Right neural foraminal narrowing. Grade 1 retrolisthesis of L5 over S1. Full report attached to pt chart, not compaired to prior imaging.

Bilateral lower extremity EMG 9/30/2022:
This is a essentially normal study providing no convincing electrophysiological evidence of large fiber peripheral neuropathy, right cervical/lumbar sacral radiculopathy or upper/lower extremity mono neuropathy to explain patient's symptoms.

Medications:
restart Gabapentin 300mg TID - increase 400mg TID
DC pregabalin 100mg caused loss of vision
DC Duloxetine 30mg qAM, ineffective
DC nabumetone, ineffective
MDP completed Oct 2022 - no relief of pain
RF Tramadol 50 mg BID
OA 8/2024
UDS C/S 08/2024 Neg

Interventional Procedures: :LEFT L4/5 LAMI PER PT IN AFRICA
- Recommend ILESI at L5-S1
8/26/24 B/L L3-5 MBB he reports no relief
10/16/2023 L3/4 ILESI no relief

Conservative Therapies:
Completed PT Sept 2022
Consider repeat FCT
DME

Referrals/Specialists:
- Cont with neurologist
- Referred to Dr. Valdes - recommended surgery; pt declined
- Continue with psych for MDD and Gen Anxiety
- Referral to new spine surgeon 10/31/24
declines new referral to spine surgeon

follow up 4 weeks.

The patient's initial evaluation was performed by a physician. Today's visit continues and is consistent with the patient's previously established plan of care and is billed incident to, as there is a doctor who is immediately available in person or via tele-communication. The supervising physician, Dr. Haddad, has reviewed the plan of care and is in agreement, including with the medications prescribed, (if applicable) for this patient.

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This note was transcribed using electronic software and voice recognition software. This note may contain inherent spelling, grammatical or dictation inaccuracies due to software. Although proofreading was performed, some errors do persist. Please feel free to contact our office with any clarification or questions.

OPIATE PAIN MEDICATIONS: The risks of opiate pain medications have been discussed with the patient. These risks include addiction, medication tolerance, physical dependence, respiratory depression, constipation, nausea, an opioid-induced hyperalgesic state (increase in pain from opioid medications) and even death. Tolerance can limit the efficacy of opioids for long-term pain management. The patient has been warned to keep their medication in a safe place, completely secured from unauthorized access by children or others in the home. The best way to store these medications is in a lockable container. While taking opiate medications patients are instructed to abstain from using alcohol. While taking pain medications, patients are cautioned about driving or operating heavy machinery while taking these medications due to the risk of impairment and motor vehicle accident. They are never to drive if they are impaired in any way. The patient has signed an opioid treatment agreement and states they are taking the medication as prescribed. Pharmacy board report reviewed today and consistent with history. At present the patient reports that the medication significantly improves their pain/quality of life and provides functional improvement. The patient provided their informed consent to continue opiate therapy as they believe that the benefits outweigh these risks.

As this patient is clearly uncomfortable today, the patient has been provided with a Toradol injection, 30 mg IM to afford some palliation of the acute pain. The patient tolerated the injection well and was discharged in a stable condition. Vital Signs Stable upon discharge.

Plan:

Treatment:
1. Lumbar radiculopathy
Refill Gabapentin Capsule, 400 MG, 1 capsule, Orally, three times a day, 30 days, 90 Capsule, Refills 0, Notes to Pharmacist: Do not dispense until 30 days after last fill, or on the last business day prior to 30 days in cases where pharmacy is closed on 30th day. Don't drink alcohol while taking this medication. .

Procedure: Lumbar IL ESI
ILESI L5/S1 WITH IVCS NO CC

Procedure: Toradol IM
30 mg IM

Referral To:
Reason:AT HOME STAR PHYSICAL THERAPY- eval and tx for lumbar radiculopathy.

Procedure Codes: J1885 INJ KETOROLAC TROMETHAMINE 15 MG

Follow Up: after esi.

Billing Information:

Visit Code:
99214 Office Visit, Est Pt., Level 4.
Procedure Codes:
J1885 INJ KETOROLAC TROMETHAMINE 15 MG.

Provider: Jina H Naaim, PA-C **Date: 03/30/2026**

Generated for Printing/Faxing/eTransmitting on: 03/30/2026 12:43 PM MDT

Medical Summary

BIZIMANA, OLIVIER
8330 N 19TH AVE, APT 2056, PHOENIX, AZ, US 85021-5287
DOB: 08/12/1982 **Age:** 43 Y **Sex:** male
Home: 623-227-6352
Work:
Cell: 623-227-6352
Email: boliverad@yahoo.com

Previous Name:

Advance Directive:

Primary Insurance: Medicare Part B
PCP:
Account Number: 34131
Race: Black or African American
Ethnicity: Not Hispanic or Latino
Preferred Language: English

Care Team:

Allergies
Substance: N.K.D.A. **Status:** Active.

Medical History
Problem List

Onset Date	Code	Name	Specify	Notes	Added On	Modified On	Modified By
	Z79.891	Long term (current) use of opiate analgesic **W/U Status:** confirmed			08/14/2024	08/14/2024	Marinelli, Brandy
	M54.16	Lumbar radiculopathy **W/U Status:** confirmed			10/04/2022	08/14/2024	Haddad, Christine
	G62.9	Neuropathy **W/U Status:** confirmed			10/04/2022	08/14/2024	Marinelli, Brandy
	M47.817	Spondylosis of lumbosacral joint without myelopathy **W/U Status:** confirmed			10/04/2022	08/14/2024	Marinelli, Brandy

Past Medical History
no medical problems

Medications
Name strength formulation, Sig: take route frequency
Refill Gabapentin 400 MG Capsule, Sig: 1 capsule Orally three times a day, Notes: Do not dispense until 30 days after last fill, or on the last business day prior to 30 days in cases where pharmacy is closed on 30th day. Don't drink alcohol while taking this medication.

Surgical History

Date	Reason
8/17/2022	lumbar discectomy at L5
05/2025	back surgery
05/2025	Lumbar laminectomy LEFT L4 and L5

Hospitalization

Date	Reason
03/2023	denies recent
2015	fall

Social History

Name	Value
Drugs/Alcohol:	(Alcohol Screen):No Did you have a drink containing alcohol in the past year?: No, Points: 0, Interpretation: Negative
Tobacco Use:	(Tobacco Use/Smoking):No Are you a: : nonsmoker

Vitals

Name	Date	Value
BP	03/30/2026	130/76
HR	03/30/2026	60
Ht	03/30/2026	72

Wt	03/30/2026	230
BMI	03/30/2026	31.19
Oxygen sat %	03/30/2026	96
Pain scale	03/30/2026	8
Wt-kg	03/30/2026	104.33
Body Surface Area	03/30/2026	2.3

Patient Encounters

Date	Time	Provider	Facility	Reason	Diagnosis
05/21/2026	03:30 PM	Naaim, Jina	Central Phoenix - Innovative Pain and Wellness	inj f/u	
05/07/2026	10:15 AM	Haddad, Christine, MD	Bell Road - Innovative Pain and Wellness	ILESI L5/S1 WITH IVCS NO CC	
03/30/2026	11:01 AM		Bell Road - Innovative Pain and Wellness		
03/30/2026	10:45 AM	Naaim, Jina	Central Phoenix - Innovative Pain and Wellness	MRI REVIEW	M54.16? Lumbar radiculopathy M47.817? Spondylosis of lumbosacral joint without myelopathy G62.9? Neuropathy Z79.891? Long term (current) use of opiate analgesic
03/09/2026	07:40 PM		Bell Road - Innovative Pain and Wellness		
02/16/2026	02:30 PM	Naaim, Jina	Central Phoenix - Innovative Pain and Wellness	Med Refill	M54.16? Lumbar radiculopathy M47.817? Spondylosis of lumbosacral joint without myelopathy G62.9? Neuropathy Z79.891? Long term (current) use of opiate analgesic
03/12/2025	10:30 AM	Naaim, Jina	Central Phoenix - Innovative Pain and Wellness	f/u	M54.16? Lumbar radiculopathy M47.817? Spondylosis of lumbosacral joint without myelopathy G62.9? Neuropathy Z79.891? Long term (current) use of opiate analgesic
10/31/2024	02:30 PM	Carpenter, Victoria	Central Phoenix - Innovative Pain and Wellness	post-operative	M54.16? Lumbar radiculopathy M47.817? Spondylosis of lumbosacral joint without myelopathy G62.9? Neuropathy Z79.891? Long term (current) use of opiate analgesic
10/21/2024	03:00 PM	Haddad, Christine, MD	Bell Road - Innovative Pain and Wellness	LESI L4/5 noIVCS, noCC	M54.16? Lumbar radiculopathy
09/09/2024	12:30 PM	Grigutis, Olena	Bell Road - Innovative Pain and Wellness	POST MBB	M47.817? Spondylosis of lumbosacral joint without myelopathy M54.16? Lumbar radiculopathy G62.9? Neuropathy Z79.891? Long term (current) use of opiate analgesic
08/28/2024	11:36 AM		Central Phoenix - Innovative Pain and Wellness	PA	
08/26/2024	04:15 PM	Haddad, Christine, MD	Bell Road - Innovative Pain and Wellness	B/L L3-5 MBBx2; no IVS; no CC	M47.816? Lumbar spondylosis
08/14/2024	01:00 PM	Haddad, Christine,	Central Phoenix -	DME pick up	

		MD	Innovative Pain and Wellness		M47.816? Lumbar spondylosis
08/14/2024	01:00 PM	Haddad, Christine, MD	Central Phoenix - Innovative Pain and Wellness	back pain	M54.16? Lumbar radiculopathy M47.817? Spondylosis of lumbosacral joint without myelopathy G62.9? Neuropathy Z79.891? Long term (current) use of opiate analgesic
04/02/2024	01:00 PM	Carpenter, Victoria	Central Phoenix - Innovative Pain and Wellness	pain, back	M54.16? Lumbar radiculopathy M47.817? Spondylosis of lumbosacral joint without myelopathy G62.9? Neuropathy
10/23/2023	02:15 PM	Carpenter, Victoria	Central Phoenix - Innovative Pain and Wellness	VER video - med refill	M54.16? Lumbar radiculopathy M47.817? Spondylosis of lumbosacral joint without myelopathy G62.9? Neuropathy
10/16/2023	11:00 AM	Haddad, Christine, MD	Bell Road - Innovative Pain and Wellness	L3/L4 ILESI no IVCS; No CC	M54.16? Lumbar radiculopathy
09/25/2023	11:00 AM	Haddad, Christine, MD	IPW Scottsdale	VER *F/U Visit - Routine	M54.16? Lumbar radiculopathy M47.817? Spondylosis of lumbosacral joint without myelopathy G62.9? Neuropathy
04/07/2023	10:46 AM		IPW Scottsdale	referral	
03/06/2023	04:45 PM	Haddad, Christine, MD	IPW Scottsdale	VER F/U POST PROCEDURE	M54.16? Lumbar radiculopathy M47.817? Spondylosis of lumbosacral joint without myelopathy G62.9? Neuropathy
03/01/2023	01:15 PM	Haddad, Christine, MD	IPW Scottsdale	bilateral S1 tfesi no sedation	M54.16? Lumbar radiculopathy
02/20/2023	04:45 PM	Haddad, Christine, MD	IPW Scottsdale	VER F/U POST PROCEDURE	M54.16? Lumbar radiculopathy M47.817? Spondylosis of lumbosacral joint without myelopathy G62.9? Neuropathy
02/15/2023	02:45 PM	Haddad, Christine, MD	IPW Scottsdale	bilateral L3-5 mbb x 2 no sedation	M47.817? Spondylosis of lumbosacral joint without myelopathy
01/30/2023	02:00 PM	Haddad, Christine, MD	IPW Scottsdale	VER *F/U Visit - Routine	M54.16? Lumbar radiculopathy M47.817? Spondylosis of lumbosacral joint without myelopathy G62.9? Neuropathy
11/17/2022	01:45 PM	Haddad, Christine, MD	IPW Scottsdale	LESI L5-S1, no IVCS, no CC	M54.16? Lumbar radiculopathy
11/08/2022	09:15 AM	Grigutis, Olena	IPW Scottsdale	VER *F/U Visit - Routine	M54.16? Lumbar radiculopathy M47.817? Spondylosis of lumbosacral joint without myelopathy G62.9? Neuropathy
10/21/2022	08:30 AM	Haddad, Christine, MD	IPW Scottsdale	VER *F/U Visit - New Imaging Review	M54.16? Lumbar radiculopathy M47.817? Spondylosis of lumbosacral joint without myelopathy G62.9? Neuropathy
10/04/2022	03:30 PM	Haddad, Christine, MD	IPW Scottsdale	VER	M47.817? Spondylosis of lumbosacral joint without myelopathy M54.16? Lumbar radiculopathy G62.9? Neuropathy

Health Maintenance

Practice Configured

Frequen

Name	Last Done	cy	Due Date	Status	Notes
Tetanus		10Y	03/30/2026	Noncompliant	
Cholesterol screening Male (Low CVD Risk)		-	03/30/2026	Noncompliant	

Referrals

Incoming Referrals

Referral From	Referral To	Start Date	End Date	Reason
Christine Haddad	Christine Haddad	02/24/2023	04/25/2023	bilateral S1 tfesi no sedation
Christine Haddad	Christine Haddad	02/14/2023	04/15/2023	bilateral L3-5 mbb x 2 no sedation
Provider Migration	Christine Haddad	02/07/2023	02/07/2024	OFFICE VISIT
Christine Haddad	Christine Haddad	11/11/2022	01/10/2023	(NVSC) LESI L5-S1, no IVCS, no CC

Outgoing Referrals

Referral From	Referral To	Start Date	End Date	Reason
Jina H Naaim		03/30/2026	03/30/2027	AT HOME STAR PHYSICAL THERAPY- eval and tx for lumbar radiculopathy.
Victoria Carpenter		10/31/2024	10/31/2025	Please send referral to Dr. Shaleen Vira, Banner Neurosurgery. He would like an opinion on surgical options for his lumbar radicular pain. He has failed lumbar steroid injections.
Christine Haddad		10/09/2024	01/07/2025	LESI L4/5 noIVCS, noCC
Christine Haddad		09/10/2024	09/10/2025	B/L L3-5 MBBx2; no IVS; no CC
Christine Haddad		08/16/2024	11/14/2024	B/L L3-5 MBBx2; no IVS; no CC
Victoria Carpenter		04/03/2024	04/03/2025	Please send referral to Dr. Mauricio Valdes, Orthopedic Spine Surgeon | Patient with chronic low back and bilat lower extremity pain. Failed lumbar ESI. He would like to know if there are surgical options to improve his pain.
Christine Haddad		10/05/2023	10/05/2024	L3/L4 ILESI no IVCS; No CC
Christine Haddad	Provider Migration	03/06/2023	03/06/2024	numbness and pain to lower extremities
Victoria Carpenter	Provider Migration	10/21/2022	10/21/2023	Patient would like a second opinion, low back pain with radicular symptoms into bilateral lower extremities. States had L5-S1 discectomy in California Aug 2022 with no relief of his symptoms.

p - pending approval